

Columbia (NYP)

NewYork-Presbyterian

Win probability: High

Installed

Expand, priority

NeuroBlate

Visualase



ACCOUNT HEALTH

ACCOUNT TYPE Installed	DOING LITT Yes	LITT SYSTEM NeuroBlate + Visualase	SEEG PROGRAM Yes
ROBOT ROSA (confirm) — SEEG-active	INTRA-OP MRI Confirm in field	NAVIGATION Confirm in field	>50 CRANI/YR Yes
SERVICE CONTRACT Confirm in field	POSTURE Expand, priority	CASES LOGGED 14	HEALTH GRADE A+ · 91/100

► Bought own NeuroBlate system in 2026 (CUMC-C-003, \$289K capital).

Fastest-growing account by revenue. McKhann runs mostly primary-tumor LITT (12 primary, 2 mets) over an enormous epilepsy, neuro-onc and referral base. McKhann is leaving for Emory; Gill is the successor. Brett Youngerman shows 2 LITT in claims not in our log (Visualase?), with an in-house Visualase rep on site.

B Business Health

HEALTH DRIVERS

CASES (3-YR)

10

1 → 8 → 1 by yr

PROBES / CASE

1.3

pull-through

\$ / CASE

\$19.0K

+4% vs region

POTENTIAL / YR

\$108K

+8 cases

Net sales by year

consumable + capital, with 2026 projection

Addressable reservoir / yr

LITT-appropriate cases by indication

Mets	22 / yr of 434 pool
Radiation necrosis	4 / yr of 76 pool
Intractable epilepsy	24 / yr of 487 pool
Recurrent GBM	0 / yr of 4 pool

P Physician Universe

PERFORMING LITT 2

Guy McKhann 9 LITT cases

Brian Gill 5 LITT cases

LITT KOLS ★ 3

★ Neil Feldstein Grade A
Co-author on Youngerman 2018 Epilepsia demonstrating MRg-LITT laser ablation is effective for temporal lobe epilepsy when hippocampal seizure onsets are SEEG-localized — direct SEE

★ Brian Gill Grade A
Directly performs LITT clinically for brain tumors (explicitly listed among his techniques alongside stereotactic navigation, radiosurgery, brachytherapy, fluorescence-guided surge

★ Guy McKhann Grade A
Direct, high-volume MRgLITT/NeuroBlate operator for both drug-resistant epilepsy and primary brain tumors. Senior/co-author on landmark SEEG-guided laser ablation TLE outcome studi

LITT-NAÏVE TARGETS 3

Raymond F Sekula 2 tumor crani

Neil Feldstein 6 SEEG · 19 epi crani

Jeffrey N Bruce confirm volume

Click a highlighted name → research appendix

REFERRERS BY INDICATION 9

Guy McKhann Primary 3

Maria Ordonez Metastasis 2

Peter CheiWay Pan Primary 2

Aya Haggiagi Primary 1

Samuel E Koszer Epilepsy pool 344

Shraddha Srinivasan Epilepsy pool 96

Michael Waldrop Rad-necrosis pool 217

Katherine D Brodie Rad-necrosis pool 206

Wen R Zhang Rad-necrosis pool 189

S SWOT Analysis

Strengths why this is a LITT account

- Owns its NeuroBlate system (2026 capital purchase) — committed installed base, not a loaner.
- Revenue trajectory is up and to the right across the last 3 years.
- Established high-volume LITT operator anchoring the program.
- Large addressable reservoirs in mets, intractable epilepsy.
- ROSA robot on site — strategic alignment with the pending Zimmer acquisition.
- Deep epilepsy referral bench feeding SEEG → ablation pathway.

Weaknesses address these

- Competitive laser in house (Visualase) splitting case flow.

Opportunities where we grow

- ~36 addressable cases/yr beyond current logged volume.
- Develop additional surgeons: Brian Gill, Raymond F Sekula, Neil Feldstein.
- Activate radiation-necrosis pathway via rad-onc referrers.
- \$108,352/yr incremental business potential in the plan.

Threats report from field

- Medtronic Visualase rep presence on site.
- Key champion departing — succession & relationship risk.
- Competitive KOL influence — confirm allegiances in field.

T Strategy & Tactics

DEVELOP SURGEONS Build the bench beyond the anchor operator

Primary targets: **Brian Gill, Raymond F Sekula, Neil Feldstein, Jeffrey N Bruce**

- Proctoring / cadaver lab with the anchor surgeon as champion.
- Co-scrub first 3–5 cases; pair with clinical specialist.
- Share indication-matched LAANTERN outcomes for their subspecialty.
- Peer-to-peer visit to a NeuroBlate reference site.

CRACK COMPETITIVE Displace Visualase on complex / repeat ablations

- Position full-perimeter monitoring and directional side-fire for complex geometries.
- Map which surgeon runs Visualase vs. NeuroBlate; convert the swing surgeon.
- Leverage in-house owned NeuroBlate capital to make ours the default.

ACTIVATE REFERRALS Convert referral reservoir into scheduled cases

- Epilepsy: SEEG-localized foci → laser amygdalohippocampotomy pathway with epileptology.
- Radiation necrosis: rad-onc tumor board presence, post-SRS surveillance → LITT.
- Build an indication-specific referral one-pager per pathway.

Research Appendix — Columbia (NYP)

Organized by physician · LITT & LITT-adjacent research relevance · click a name on the card to jump here

Brian Gill

LITT relevance A

Neurological Surgery — Columbia Oral Maxillofacial Surgery Associates, Saint Charles Hospital

Neurosurgical oncologist & physician-scientist at Columbia/NewYork-Presbyterian (CUIMC); MSKCC-trained, former Columbia chief resident; complex brain tumors, awake mapping, and LITT — McKhann's likely LITT successor.

- Glioma neurosurgical oncology
- LITT / laser interstitial thermal therapy
- Awake/asleep brain mapping in eloquent gliomas
- Peritumoral microenvironment & tumor-neuron interactions
- Glioma-associated (tumor-related) epilepsy
- Stereotactic navigation & radiosurgery
- Brain metastases & skull base tumors
- Blood-brain barrier disruption for glioblastoma

Why they matter: Directly performs LITT clinically for brain tumors (explicitly listed among his techniques alongside stereotactic navigation, radiosurgery, brachytherapy, fluorescence-guided surgery). Full neuro-oncology profile — glioma/GBM, brain metastases, skull base — plus stereotactic and awake-mapping expertise makes every ablation indication convertible. Deep glioma-associated seizure research and direct co-authorship with Columbia LITT KOL Guy McKhann. Critical to secure as McKhann's LITT successor before his Emory departure.

KEY & RECENT PUBLICATIONS

Single unit analysis and wide-field imaging reveal alterations in excitatory and inhibitory neurons in glioma (Brain)

2022 — Glioma-associated epilepsy; peritumoral electrophysiology (w/ McKhann, Canoll, Schevon)

The clinical and neurocognitive functional changes with awake brain mapping for gliomas invading eloquent areas

2023 — Awake brain mapping / eloquent glioma resection

Glioma-induced alterations in excitatory neurons are reversed by mTOR inhibition

2024 — Tumor-neuron interaction; glioma-associated epilepsy mechanism

COLLABORATORS & KOL NETWORK

Guy M. McKhann II · Peter Canoll · Catherine A. Schevon · Jeffrey N. Bruce

[Institutional profile](#) ➤ [Medscout](#) ➤

Guy McKhann

LITT relevance A

Neurological Surgery — Columbia Oral Maxillofacial Surgery Associates, Newyork Presbyterian Columbia University Irving Medical Center

Professor & Director of Epilepsy/Movement Disorder Surgery, Columbia/NewYork-Presbyterian (moving to Emory as Chair of Neurosurgery); high-volume epilepsy + glioma surgeon, NeuroBlate LITT user.

- MR-guided LITT for epilepsy & tumors
- SEEG-guided seizure localization & robotic stereotacty
- Temporal lobe epilepsy / selective laser amygdalohippocampotomy
- Human microelectrode neurophysiology of seizure onset
- Awake & asleep brain mapping
- Low- and high-grade glioma resection
- Brain metastasis surgery
- Epilepsy-surgery society leadership

Why they matter: Direct, high-volume MRgLITT/NeuroBlate operator for both drug-resistant epilepsy and primary brain tumors. Senior/co-author on landmark SEEG-guided laser ablation TLE outcome studies and multicenter LITT cohorts; presents on LITT at AES; directs epilepsy surgery program and co-directs neuro-oncology. Textbook Grade A ablation KOL.

KEY & RECENT PUBLICATIONS

Laser ablation is effective for temporal lobe epilepsy if hippocampal seizure onsets are localized by SEEG (Youngerman, McKhann; Epilepsia)

2018 — SEEG-guided LITT selective amygdalohippocampotomy outcomes

MR-guided laser interstitial thermal therapy (MRgLITT) review (Neurosurgery)

2016 — LITT technique/indications review

Microphysiology of epileptiform activity in human neocortex (Schevon, McKhann et al.)

2008 — Human microelectrode seizure-onset neurophysiology

State of the art: glioma-associated epilepsy—bridging tumor biology and epileptogenesis

J Neurooncol · 2025 — glioma-associated epilepsy

Brain tumor-related epilepsy management: SNO consensus review

Neuro-Oncology · 2023 — tumor-related epilepsy (incl. LITT options)

Long-term outcomes of mesial temporal LITT for drug-resistant epilepsy: a multi-centre cohort

J Neurol Neurosurg Psychiatry · 2023 — LITT long-term epilepsy outcomes

Glioma-induced alterations in excitatory neurons are reversed by mTOR inhibition

Neuron · 2024 — glioma / tumor-related epileptogenesis

COLLABORATORS & KOL NETWORK

Brett Youngerman · Catherine Schevon · Neil Feldstein · Sameer Sheth · Eric Leuthardt · Veronica Chiang · Patrick Landazuri

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Neil Feldstein

LITT relevance A

Neurological Surgery — Columbia Oral Maxillofacial Surgery Associates

Director of Pediatric Neurosurgery, Columbia University / NewYork-Presbyterian (Morgan Stanley Children's Hospital), NY; pediatric epilepsy, Chiari, tumors.

- Pediatric epilepsy surgery
- SEEG (stereoelectroencephalography)
- MRg-LITT laser ablation for MTL
- Hemispherectomy
- Chiari malformation
- Craniosynostosis / endoscopic surgery
- Pediatric brain & spinal cord tumors

Why they matter: Co-author on Youngerman 2018 Epilepsia demonstrating MRg-LITT laser ablation is effective for temporal lobe epilepsy when hippocampal seizure onsets are SEEG-localized — direct SEEG-guided LITT work. Also senior co-author on Columbia's pediatric SEEG focal-epilepsy series. Runs one of the Northeast's largest pediatric epilepsy-surgery programs offering SEEG, LITT and DBS; direct co-authorship with LITT KOLs Youngerman, McKhann, and Sheth.

KEY & RECENT PUBLICATIONS

Laser ablation is effective for temporal lobe epilepsy if hippocampal seizure onsets are localized by SEEG
2018 — SEEG-guided MRg-LITT laser ablation for MTL

Safety and efficacy of SEEG in pediatric focal epilepsy: a single-center experience
2018 — Pediatric SEEG epilepsy surgery

COLLABORATORS & KOL NETWORK

Brett Youngerman · Guy M McKhann · Sameer A Sheth · Guy McKhann · Sameer Sheth

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